



Policy

SCGOPHCG Clinical Handover - Communicating for Safety

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Introduction

Clinical handover is an explicit transfer of information supporting the transfer of clinical accountability and responsibility between healthcare professionals to enable continuity of patient-focussed, safe and high-quality care via a number of formats¹.

SCGOPHCG is committed to ensuring timely, relevant and structured clinical handover that maintains a safe environment and continuity of patient care at SCGOPHCG.

The requirements of this Policy are supported by the [National Safety and Quality Health Service Standard 6: Communicating for Safety](#).

This policy should be read in conjunction with:

- [WA DOH Clinical Handover Policy \(MP0095/18\)](#)

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☒
Other:	☐		

1.0 Clinical Handover Principles

- Clinical handover must occur whenever responsibility of the patient is changed from one clinician (includes medical, nursing, midwifery and allied health staff) to another, both internal and external to the organisation and should occur at the time of or prior to, the transfer of responsibility.
- Clinical handover must be understood as an explicit transfer, not just of information, **but of clinical accountability and responsibility**.
- Clinical handover is to occur with the patient³ and patient's multi-disciplinary healthcare team at the bedside where practicable^{4,5,6}.
- Staff must consider cultural, gender diversity, ethical, communication and safety needs and preferences⁴
- All clinical handovers (other than discharge) must use the iSoBAR process to guide the content and structure of the clinical handover, in a manner that suits the clinical context.
- Clinical handover should be conducted face-to-face wherever possible. eReferral is not an acceptable method of providing clinical handover.
- Voice recorded handovers, SMS and other social media platforms are not permissible within WA Health¹.
- All clinical incidents relating to a failure to clinically handover a patient are notified and managed in accordance with the WA DoH Clinical Incident Management Policy²
- Staff members handing over must be able to give comprehensive information and clarify aspects of care if required regardless of the modality of handover
- All SCGOPHCG forms related to clinical handover must comply with the mandatory elements of the policy and must be endorsed by the Communicating for Safety Committee.

All clinical staff must receive education on clinical handover process and this policy. It is recommended that this occurs at the commencement of rotation or employment, and also following policy revision.

Effective clinical handover does not guarantee quality care – the responsibility and information handed over still needs to be acted upon. Evidence indicates that ineffective clinical handover can lead to adverse events such as:

- incorrect treatment,
- delays in diagnosis and treatment,
- increased length of stay,
- increase in expenditure,
- patient complaints,
- malpractice claims.

1.1 Patient Prioritisation for Clinical Handover

Patients should be handed over according to their clinical situation and clinical risk.

Clinical handover should cover all patients, for which responsibility is being handed over, identifying any:

- patients of concern,
- newly admitted patients,
- patients requiring a transfer of some or all aspects of care.

For patients of concern, the use of Information and Communications Technology (ICT) tools alone should not be considered effective clinical handover.

1.2 Clinical Handover Process

Clinical handover content should be clear, concise, and use easily understood words with minimal use of abbreviations. Using iSoBAR (Appendix 1) to guide clinical handover will ensure that:

- The information presented is relevant to that which is necessary to provide safe care to the patient.
- All clinicians involved have an opportunity to discuss the management plan, clarify information, and ask and respond to questions.
- Verification of understanding occurs.
- Responsibilities and planned actions are clearly understood.

1.3 Appropriate Modality

- Face-to-face clinical handover provides the receiver with aspects of communication, such as body language, and includes the opportunity to clarify information.
- Clinicians escalating care of deteriorating patient / patient of concern are to use the iSoBAR format to facilitate effective communication of information. Telephone communication is an accepted modality but must be followed by face-to-face communication (i.e. once the notified clinician attends the ward/area/department to review the deteriorating patient).
- Taped and/or voice recorded clinical handovers are not permitted under any circumstances.

1.4 Appropriate Environment

- Periods of clinical handover should be in place to limit non-critical interruptions to communication during clinical handover.
- Where practicable, nursing shift clinical handovers should be conducted, in the presence of the patient and / or carer (at the bedside). This facilitates participation of patient and / or carer in clinical handover communication if desired.

1.5 Supporting Documentation

Clinical handover content should be clear, concise, and contain easily understood words with minimal, accepted abbreviations.

All clinical handovers must be supported by appropriate endorsed SCGOPHCG documentation, these may include mobile electronic tools or computer-generated patient information sheets or use agency specific referral forms (e.g. Silver Chain, Rehabilitation in the Home [RITH], Transitional Care Placement [TCP]).

2.0 Inter Professional / Multidisciplinary Team Handover (e.g. journey board patient huddle)

Where inter professional / multidisciplinary team clinical handover occurs, the iSoBAR format is to be utilised.

2.1 Journey Board

The traffic light system should be utilised by the multidisciplinary team to indicate the patient's 'readiness for discharge' against the listed discharge destination.

- **Red** – not ready for discharge
- **Amber** – nearly ready for discharge
- **Green** – ready for discharge

The journey board should provide pertinent patient information 'at a glance' and be:

- utilised on all inpatient multi-day ward areas, critical care, short stay and acute assessment units should utilise a board appropriate to their area.
- reviewed by ALL members of the multidisciplinary team involved in the patients care.
- updated daily.
- updated when the patient's condition changes.

2.2 Journey Board Huddle

The patient and next of kin/carers must be informed of any change to their care plan or EDD as a result of the journey board meeting. Communication with family/carers should be documented in the Digital Medical Record (DMR).

Journey board huddle should be ward-based, brief, held daily and include members of the multidisciplinary team involved in the patients care and may include:

- focus on patients who are nearing discharge,
- identify barriers to discharge,
- ensure referrals have been acted on,
- define suitable patients for transfer to other general hospital sites,
- allocate responsibility for a team member to notify the patient of their EDD and update communication board in patients' room,
- refer any complex discussion to the MDT meeting,
- update the EDD on journey (white) board to allow the ward clerk to update WebPAS.

2.3 Multidisciplinary Team (MDT) Meetings

It is expected that all multiday specialties have at minimum a weekly multidisciplinary team meeting. It is recommended the MDT meeting follow the journey board meeting format, and include members of the multidisciplinary team involved in the patients care and should:

- discuss all admitted multi day in-patients with priority given to patients on specialty (home) ward;
- ensure a comprehensive clinical management plan is discussed and documented;
- discuss discharge options, including suitable patients for transfer to other general hospital sites,
- identify EDD and identify barriers to discharge, ensure investigations and eReferrals are completed

2.4 Ward Rounds

Daily Consultant/Registrar Ward Rounds

- Consultants/registrar available to review patients on a daily basis in each specialty.
- Patients will be seen on a clinical priority basis;

- clinical deterioration
- new admission
- discharge pending
- On each review the consultant/registrar will lead the discussion and ensure an appropriate clinical management plan is documented including:
 - clinical investigation results,
 - altered clinical parameters,
 - pending investigations,
 - estimated date of discharge (EDD),
 - Outpatient follow-up requirements.

It is acknowledged that some patients will not require to be seen by their consultant daily e.g. those with a *comprehensive* management plans (which are updated and reviewed daily) or planned EDD.

3.0 Temporary Clinical handover (continuous and non-continuous)

Temporary clinical handover involves the process of clinical handover between shift to shift (continuous), day to day, after-hours and weekends / public holidays (non-continuous).

- Face to face clinical handover should occur whenever possible.
- Appendix [5](#) outlines the requirements for temporary clinical handover for all disciplines.

Discipline specific processes related to temporary clinical handover:

Allied Health

- For disciplines with continuous shift coverage, face to face verbal clinical handover will occur supported by written iSoBAR clinical handover documented in the medical record.
- For disciplines with non-continuous shifts, and clinical handover for weekend shifts, a written iSoBAR Clinical Handover will be documented in the medical record.

Medical

- Clinical handover of patients to medical staff can occur under multiple circumstances and should follow the same principles of timely communication and using the iSoBAR format.
- Written handover can be provided using the Medical Handover form (SCGH), iSOFT Clinical Manager (IcM) or in the Healthcare Record with the same components covered.

3.1 Shift to shift handover

Shift to shift handovers must follow all principles of clinical handover and must be conducted in the ISOBAR format.

3.2 Specialty to specialty handover

Prior to handover of clinical care, a referral / request for consult must have been made to the receiving specialty. This should be verbal and followed up by a written referral if requested (i.e. e-referral). After acceptance of care, there must be written acknowledgement of acceptance of responsibility by the receiving team in the medical record.

3.3 Critical care to ward handover

Patients transferred from areas of critical care i.e. Intensive Care Unit (ICU), Coronary Care Unit (CCU) and Theatre to a ward setting require an escort, verbal, and written handover. A written handover on the Nursing Assessment record (NAR) is required for the transfer of non-escorted patients from the Emergency Department.

3.4 Afterhours and Weekend

It is expected that for all patients of concern (POC) the responsible team will document a clear management plan and provide a verbal clinical handover.

3.5 Night Duty handover

At SCGH:

Night duty handover attendance is compulsory for all medical staff allocated to the after-hour's roster and incoming medical staff on night duty. It takes place every night between 2200 and 2230.

Under the guidance of Charlies After-Hours Team (CAT) registrar, evening interns and residents will systematically and comprehensively handover relevant and significant clinical information to the night residents, with the After-hours CNS present. The order of review is:

- Acutely unwell / sick patients (patients of clinical concern),
- Urgent pending blood/radiology results, which will change a patient's management,
- Any significant change to the management plan for a patient during the evening shift,
- Need for particular tests during the night shift (such as APTT, drug levels, blood cultures, pre op bloods etc.),
- Patients who have been unwell during the evening shift but currently stable.

Patients of clinical concern reviewed overnight, need to have both written documentation and be verbally handed over to the treating team in the morning. CAT night doctors are responsible for contacting the treating team during morning changeover.

At OPH:

The After-Hours Medical Officer will meet with the After Hours CNS and NM at approximately 1600hrs Monday – Friday, 0830hrs Saturday – Sunday / Public Holidays and at approximately 2230 hours every night to discuss the status of inpatients. This discussion will include any patients of concern, and new admissions, transfers and discharges.

3.6 Nursing handover

- Inpatient general wards utilise (iCM) nursing handover to facilitate shift to shift clinical handover.
- All nurses are responsible for updating iCM in preparation for change of shift.
- Nursing shift clinical handover is comprised of two parts:
 - Patient of concern (POC) briefing or Stand up clinical handover, and
 - Bedside clinical handover.
- Nursing staff are to sign the Nursing Care Plan to confirm receipt of clinical handover.
- Nursing clinical handover is performed at the bedside only in the Emergency Department (ED), Procedural Areas and Operating Theatre Suite.

3.7 Patient of Concern (POC) briefing or Standup clinical handover

This process is both verbal and interactive with provision for clarification of patient information and any other issues identified during the briefing.

Inpatient general ward nursing shift coordinator will:

- verify the iCM information provided by individual nursing staff and receive verbal update regarding any POC, shift coordinator to indicate clinical POC using icon.
- Commence the ≤10-minute briefing to all oncoming staff in a designated area which is private and free from distraction.
- Lead the POC briefing highlighting newly admitted patients, clinical deterioration/concerns, pending discharges, procedures, falls and pressure risks, infectious status, communicable diseases, Goals of Patient Care, psychiatric issues, social/family issues, and any other sensitive information is outlined here and not at the patient's bedside.

3.8 Bedside clinical handover - every patient, every shift, every time

Bedside clinical handover provides the opportunity to discuss and clarify any clinical information that is required. Patient confidentiality is essential to effective clinical handover and relationship development. Diagnostic information is not required to be discussed in this environment as a matter of information sharing as it is written on the clinical handover sheet.

Patients and their carers are to be invited to be involved. Ask the patient or carer if they have any comments or questions about their care. It is not to be assumed that a patient will want a carer / visitor to be present during clinical handover; confirm with patient prior to commencing clinical handover. If the patient requests privacy or is unable to give permission, the carer / visitor should be asked to leave.

All bedside clinical handovers should follow the iSoBAR format (Appendix 1)

- I – Identify
- S – Situation
- O – Observation
- B – Background
- A – Agree to a plan/Assessment
- R – Readback/Recommendation

Bed side charts should be accessed during handover to ensure the following:

- Check UMRN identification labels are present and accurate i.e. patient name, DOB
- Review risk assessment tools (e.g. Falls, Pressure Injury, Delirium, Malnutrition) and discuss changes to clinical care, discuss current / future equipment requirements,
- Check medication chart and identify patient allergies - ensure any outstanding issues resolved with delegated responsibility,
- Review observation chart/s and discuss management of any clinical escalation;
- Check intravenous device/s insertion site, including date and time, for peripheral devices complete (PIVAS),
- Check intravenous orders, infusion and lines – ensure all documents signed appropriately,

Staff providing and receiving bedside clinical handover are required to sign the Nursing Care Plan or department specific flow sheet / clinical pathway to acknowledge completion

of the clinical handover process.

If the patient is not on the ward during bed side clinical handover, the clinical handover and review of documents should still occur. If the patient is attending an offsite appointment and the 'end of bed notes' are not available, the clinical handover of care and responsibility should be documented in the medical record upon patients returns.

The following communication behaviours assist with building a therapeutic relationship between a nurse and their patient.

- A – Acknowledge the patient and carer and establish appropriate salutation
- I – Introduce yourself and your colleague.
- D – Duration of your current role in delivering care to the patient
- E – Explain what is happening and when you will come back
- T – Thank you

3.9 Patient transfer between departments for a test or appointment within SCGH or OPH

- Nurse Escort and clinical handover is required as clinically indicated, (SCGH Appendix 2 & OPH Appendix 3) or if the patient has been within the Senior Nurse Review criteria on the Observation and Response Chart (ORC).
- If a patient is escorted by a clinical staff member, a verbal clinical handover must occur between the accompanying clinician and an appropriate clinician/s at the receiving department.
- If there is no clinical staff in the receiving department, the escort must remain with the patient
- Patient clinical handover / transfer is not to be viewed solely as a process to check the completion of the checklist, but as the transfer of professional responsibility and accountability for some or all aspects of care for a patient to ensure continuity of observation and clinical management.
- Inpatients attending outpatient clinic (i.e. Ophthalmology, Ear Nose and Throat) appointments:
 - Urgent consultation (required within 24 hours) will be handed over verbally to the specialty on-call Registrar or responsible Consultant. The consultation requires an eReferral to be generated. A profession specific written clinical handover shall be completed and stored in the healthcare record.
 - .

3.10 Patient transfer from OPH to SCGH for a test or appointment.

- A verbal handover should occur either at the time of transfer, or prior to transfer.
- This should be supported by the completion of a transfer document, Nursing Clinical Handover form.
- Nurse Escort is required as clinically indicated (see Appendix 4.)

Refer to Appendix 5 for the Clinical Handover Matrix which identifies recommended delivery of handovers (e.g. face to face, telephone or written) for different situations where there is a transfer of clinical accountability and responsibility.

4.0 Permanent (transfer of care) to another ward

Clinical handover (transfer of care) is required when patients are permanently transferred between specialties or ward.

Allied Health

- Profession specific written handovers will be stored in the medical record.
- Verbal and written clinical handover will be used for patients of concern.

Medical

- Documentation of medical clinical handover is required to be completed prior to transfer, for all transfer of care between specialties within the hospital.
- The accepting consultant should be notified of all transfers in a timely manner.

Nursing

- Transfers between inpatient wards within OPH and within SCGH (including patients staying overnight in the transit lounge) require the completion of the Nursing Clinical Handover form by the transferring ward nurse and an escort of the patient with the medical record to the receiving ward.
- Phone clinical handover is not required to transfer a patient between inpatient wards on the same site.
- Patient details must be entered into Enterprise Bed Management (EBM) at the time of referral to enable decision making on room and staff allocation.
- The nurse receiving the patient must sign the nursing clinical handover acknowledging the patient and clinical handover has been received.
- If the patient is transferred to a second location within the same shift, the original document can be used with any additional information added and countersigned by the transferring nurse.

Patients transferring from SCGH to OPH:

- Transfers between SCGH to OPH require the transferring nurse to call the receiving ward to provide a verbal handover prior to the patient's departure.
- Completion of the Nursing Clinical Handover and transfer of healthcare records. (Appendix 7)
- Communication with the patient's family / carer should occur before transfer.
- A NaCS summary (no medication list) can be used only for patients transferring from SCGH to OPH.
- A current medication chart must accompany the patient.
- Please also refer to SCGOPHCG Procedure "[Transfer of Patients from SCGH to OPH](#)"

Patients transferring from OPH to SCGH:

- Same process as above (SCGH to OPH) (Appendix 7)
- Refer to OPH Policy "[Patient Transfers – to and from External Facilities](#)"
- Refer to (SOP) SCGH Read OPH Write Interhospital Transfer and Critical Co used Forms SCGOPHCG.
- In an emergency situation the nursing clinical handover is not required. Inpatient admission paperwork along with nursing / medical escort and verbal (face to face) clinical handover is sufficient.

4.1 Emergency Department transfer to inpatient ward

Emergency Department (ED) clinical handover to admitting units/wards will be performed at the bedside if clinically appropriate.

4.1.1 Unescorted transfer from the emergency department to inpatient ward

- The nurse responsible for the patient's care in ED will complete a written iSoBAR clinical handover, documented on the Emergency Department Nursing Assessment Record. The name and designation of the ED nurse

completing the written handover and the receiving nurse in the clinical ward/unit is to be recorded in the iSoBAR section of Emergency Department Nursing Record.

4.1.2 Escorted transfer from the emergency department to inpatient ward and operating theatre:

- ED nurse escorting the patient gives face-to-face handover going through the documentation to the receiving nurse using iSoBAR format.
- The name and designation of the ED nurse completing the written handover and the receiving nurse in the clinical ward/unit is to be recorded in the iSoBAR section of Emergency Department Nursing Record.

4.2 Patient transfer to general inpatient ward from Post Anaesthetic Care Unit

All post anaesthetic patients discharged from the Post Anaesthetic Care Unit (PACU) require a nurse escort who must receive a face-to-face clinical handover using iSoBAR format. Documentation of the name/s and designation of the nurses' giving and receiving clinical handover will be recorded in the Post Anaesthetic Care Unit record.

4.3 Direct transfer from Theatre or PACU to ICU

Patients transferring directly to ICU from theatre require:

- an escort with an Anaesthetist, providing a verbal (face to face) handover to the receiving clinicians.
- prior to the transfer, a telephone clinical handover of the patient by the theatre nurse to the ICU nursing shift co-ordinator or delegate.

Patients transferring directly to ICU from PACU are:

- transferred back to ICU escorted by an ICU nurse who will attend PACU when requested to receive handover.
- The ICU nurse can request an escort (if uncomfortable to escort alone) by ICU medical team, ICU nursing team, Anaesthetics or PACU.

5.0 Temporary (transfer of care) to another area

5.1 Direct transfer from Ward / DOSA to Theatre

At SCGH:

Patients being transferred to theatre are to be accompanied by a nurse escort and require a completed Perioperative Record. The nurse is required to provide a verbal clinical handover using iSoBAR principle and is to remain with the patient until the holding bay nurse (during hours) or the PACU nurse (after hours) has checked all aspects of the Perioperative Record and patient's preparation.

The nurse escorting the patient is to sign the Perioperative Record. Refer to NPG#55 Pre-Operative Care for pre-operative information.

At OPH:

A nurse is only required to escort patients to theatre if they meet the Escort Criteria (Appendix 6). The anaesthetic nurse will check all aspects of the Peri-operative Checklist and patient's preparation. See Appendix 3 Clinical Handover Escort Matrix OPH

6.0 Permanent transfer to another facility

All transfer of care should be formally handed over at time of, or prior to, transfer. Handover should follow ISOBAR format and include:

- pertinent clinical information
- past and current treatment, including procedures, therapies and dates performed,
- significant diagnostic test results and test results pending,
- care plans / management plans including risk assessments.

Completion of a verbal handover should be documented in the medical record with contact details of at least one of each providing and receiving clinicians.

In addition to a verbal component, inter-facility transfers must involve a detailed transfer document or discharge summary. This document should arrive with the patient.

Nurse or Medical escort is generally not required with the exception of patients transferring from a critical care area to another HSP critical care area.

6.1 Allied Health

- Profession specific written handovers will be stored the medical record with a copy forwarded to the outpatient service/ external facility / community agency according to department specific timeframes.
- For agencies requiring specific referral forms (e.g. RITH, TCP, Silver Chain), these will be completed instead.
- Profession specific written handover into NaCS for GP actions when indicated
- Verbal handover will be provided for patients requiring urgent follow-up (within 24 hours) or for complex patients and patients of concern.

6.2 Medical

Patients transferred to another health care facility (excluding Osborne Park Hospital), and Home Link must have a completed NaCS Discharge summary.

6.3 Nursing

Patients transferred to another health care / residential care facility, outside of Sir Charles Gairdner Osborne Park Health Care Group, require the completion of the Nursing Transfer Summary and Discharge / Transfer Checklist (envelope).

For patient transferred across sites only require the Nursing Clinical Handover as the Healthcare record are transferred along with the patient.(Appendix 7)

All transfers irrespective of facility require a phone call from the discharging ward to the receiving ward.

7.0 Radiology

The Radiology request form and the validated Radiology report are written documents that address clinical handover requirements of iSoBAR tool.

8.0 Discharge

Patients discharged from the Emergency Department are to have a completed department specific electronic discharge summary (i.e. NaCS ED).

For all inpatients, the doctor of the discharging team is responsible for generating a NaCS discharge summary incorporating the entire inpatient episode, regardless of episode

being outside of their specialty group.

Patients admitted for less than 24 hours may not require a NaCS discharge summary. Refer to SCGOPHCG Hospital Policy “Discharge of Patient for exceptions”.

NaCS (WA Health Notifications and Clinical Summaries) to include details of:

- Primary and other relevant diagnoses (those that required investigation or intervention, increased length of stay, increased nursing care or complicated treatment plans and/or discharge),
- Treatment course to date including relevant procedures, relevant diagnostic test results and test results pending,
- Blood transfusion if occurred during hospital stay,
- Discharged medications including changes to medications,
- pressure injury assessments,
- falls risk assessment, infectious risk assessment,
- Outstanding outpatient and medical appointments,
- Ongoing and follow-up plans with responsibilities assigned to specific professions

All inpatients discharged from SCGOPHCG require the completion of the Discharge / Transfer Checklist envelope. It is the ward nurse’s responsibility to complete the final check of the envelope and contents.

General Practitioners will be provided a complete and accurate discharge summary within two days of discharge. A copy of the discharge summary is to be placed in the discharge envelope provided to the patient at the time of discharge.

8.1 Patient transfer to Transit Ward awaiting discharge home.

No nurse escort or verbal handover required.

Patients that are for discharge home must have:

- Discharge/Transfer checklist envelope commenced
- Completed or commenced NaCS Discharge summary
- Outstanding care that requires completion prior to discharge documented in the Digital Medical Record (DMR)
- Current Admission paperwork to accompany patient – (HSA to handle transport of Healthcare Record)

It is the transit ward nurses’ responsibility to review the Discharge / Transfer Checklist and make the final discharge entry in the DMR.

9.0 Patients referred to community services (Temporary or Permanent)

- Clinicians consulting in the ambulatory setting will provide a verbal handover to community services for patients requiring urgent follow-up (within 24 hours).
- Following an Outpatient event, a profession specific written clinical handover will be provided to the community service and a copy shall be retained in the Healthcare Record
- For agencies requiring specific referral forms (e.g. RITH, TCP, Silver Chain), these will be completed instead.

Compliance, monitoring and evaluation

The SCGOPHCG Communicating for Safety Committee is responsible for the compliance, monitoring and evaluation of this policy. See: NSQHSS Audit Schedule

SCGOPHCG Clinical Governance Committee will ensure the ongoing

evaluation of the Communicating for Safety Committee to ensure opportunities for improvement are identified, actioned, and the outcome of interventions assessed.

Related Policies/Documents

- [National Safety and Quality Health Service Standard 6: Communicating for Safety](#)
- [DOH Clinical Handover Policy \(MP0095/18\)](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [SCGOPHCG Discharge of Patient Policy](#)
- SCGH Read OPH Write Interhospital Transfer and Critical Co-use Forms SCGOPHCG.

References

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Applicable Standards	NSQHS Standards: Std 1: Clinical Governance  Std 6: Communicating for Safety 				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Appendix 1: iSoBAR

i	Identify	Identify and introduce yourself – including location, designation, date and time; confirm the patient's identity using at least three patient identifiers
S	Situation	Briefly state the principle problem, what, when, how severe
o	Observation	Most recent vital signs, physical assessment
B	Background	Information related to the patient, what has changed that is requiring you to escalate – medication, fluid, test results
A	Agree to a plan / Assessment	Given the situation what has happened / needs to happen? What are you expecting (advice, orders, review, transfer) What is the level of urgency
R	Readback / Recommendation	Clarify and check for shared understanding. Who is responsible for what and by when



Appendix 2: Clinical Handover Escort Matrix SCGH

Department	Checklist	Medical Records	Escort	Clinical Handover	RTW escort
Invasive Intervention Department					
NIIS _{WA}	NIIS _{WA}	√	√	√	√
Cardiovascular Invasive Lab	CVIL	√	√	√	√
G75	Procedural Checklist	√	√	√	√
RDU - Interventional Radiology	Procedural Checklist	√	√	√	√
Operating Theatre / PACU	Theatre	√	√	√	√
Procedural / treatment (invasive) departments					
Radiation Oncology (Brachytherapy)	x	√	√	√	√
Haematology / Oncology	x	√	√	√	√
Dialysis	x	√	√	√	√
Radiology (CT/MRI)	CT Consent Checklist & MRI Safety Checklist	√	Senior Nurse Review	Senior Nurse Review	x
Nuclear Medicine	NUC MED and PET	√	Senior Nurse Review	Senior Nurse Review	x
Procedural / treatment (non-invasive) departments					
Radiology (U/S X-ray)	x	√	Senior Nurse Review	Senior Nurse Review	x
Radiation Oncology	x	√	Senior Nurse Review	Senior Nurse Review	x
Outpatient Department	x	√	Senior Nurse Review	Senior Nurse Review	x

*Patients being transferred to departments where clinical support staff are not available, will require the nurse to escort and remain with the patient.

* Senior Nurse Review includes patients under senior nurse escalation or above or if a nurse requires guidance to determine if an escort is appropriate. All cardiac monitored patient must be escorted by an ALS trained clinician.

Appendix 3: Clinical Handover Escort Matrix OPH

Department	Checklist	Medical Records	Escort **	Clinical Handover	RTW escort
Invasive Intervention Department					
Procedural room - Endoscopy	Gastroenterology Procedure form	√	x	x	√
Procedural room – Cystoscopy	Local procedure	√	x	x	√
Operating Theatre	Theatre	√	x	x	√
PACU	Theatre	√	√	√	√
Procedural / treatment (invasive) departments					
Radiology: Interventional Procedures	x	√	Senior Nurse Review	√	Senior Nurse Review
Procedural / treatment (non-invasive) departments					
Radiology (CT, U/S X-ray)	x	√	Senior Nurse Review	Senior Nurse Review	x
Outpatient Department	x	√	Senior Nurse Review	Senior Nurse Review	x

**** If the patient meets requirements in Appendix 6 a nurse escort is required**

***Patients being transferred to departments where clinical support staff are not available, will require the nurse escort to remain with the patient.**

*** Senior Nurse Review includes patients under senior nurse escalation or above or if a nurse requires guidance to determine if an escort is appropriate.**

Appendix 4: Clinical Handover Escort Matrix for patients being transferred from OPH to SCGH for procedure.

Department	Checklist	Medical Records	Escort	Clinical	RTW escort
Invasive Intervention Department					
G75	Procedural Checklist	√	Senior Nurse review	√	Senior Nurse review
RDU - Interventional Radiology	Procedural Checklist	√	Senior Nurse review	√	Senior Nurse review
Procedural / treatment (invasive) departments					
Radiation Oncology	x	√	Senior Nurse review	√	Senior Nurse review
Dialysis	x	√	x	√	x
Radiology (CT/MRI)	CT Consent Checklist & MRI Safety Checklist	√	Senior Nurse review	√	Senior Nurse review
Nuclear Medicine	NUC MED and PET	√	Senior Nurse review	√	Senior Nurse review
Procedural / treatment (non-invasive) departments					
Radiation Oncology	x	√	Senior Nurse review	√	Senior Nurse review
Outpatient Department	x	√	Senior Nurse review	√	Senior Nurse review

*Clinical handover should occur

*Nurse Escorts are generally not required for patients transferred from OPH to SCGH for test or appointment.

* Senior Nurse Review includes patients under senior nurse escalation or above or if a nurse requires guidance to determine if an escort is appropriate.

Appendix 5: Clinical Handover Matrix

WHY implement standard		In order to provide optimal patient care by accurately handing over clinical information and ensuring that responsibility and accountability of each patient's care is clearly defined.				
HOW should clinical information be handed over?		Clinical information should be handed over in an iSoBAR structure to ensure that the pertinent information is included in each handover.				
WHO should be involved in handover?		Key participants in the handover process should be identified and available to attend the handover of their patients. Patients, carers and family members are included in clinical handover, where appropriate.				
WHEN should handover occur?		Escalation of a deteriorating patient	Patient transfers (to another ward, facility or to / from community)	Patient transfers (for a test/appointment)	Shift to shift change over	Team handover (including multidisciplinary)
WHAT Is recommended when delivering handover?	Face to face plus written	***	***	***	***	***
	Telephone plus written	**	***	***	**	**
	Face to face or Telehealth or videoconference only	**	**	**	**	**
	Supplemental Tools ¹ plus telephone	*	*	**	*	*
	Telephone only	*	*	*	*	*
	Supplemental Tools ¹ only	*	*	*	*	*
	Written only	*	*	*	*	*

LEGEND

***	Recommended Options	**	Adequate	*	Not recommended
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¹ Supplemental Tools able to be utilised to supplement clinical handover include computer and electronic applications such as:

- iCM Clinical Manager
- Electronic Whiteboards
- eReferral
- BOSSNET

Tools unacceptable for Clinical Handover include:

- Recording devices (video and voice)
- SMS
- Other social media platforms

Version	Effective from	Effective to	Amendment(s)
1.0	October 2018	October 2021	Original version
2.0	January 2019	December 2022	Inclusion of patient/carers involvement

Appendix 6: Criteria for Clinical Personnel Escort/Transfer at OPH

The following patients require an assessment by the Duty Nurse Manager/CNM or Shift Coordinator in order to determine if clinical personnel (nurse, midwife or medical officer) are required to escort the patient.

- If patient requires senior nurse review as per RRAD policy
- Unconscious patients, received a pre-medication or is sedated
- Patients with potential for airway compromise or requiring suctioning
- Patients requiring continuous oxygen therapy
- Acutely unstable patients or with high acuity
- Epidural or regional infusions in progress
- IV opioid infusion or Patient Controlled Analgesia in progress
- IV infusions in progress
- Sedation score of **2 or 3** after receiving sedation
 - **Sedation Score**
 - **0 – wide awake**
 - **1 – easy to rouse**
 - **2 – easy to rouse but constantly drowsy**
 - **3 – somnolent and difficult to rouse**
- A Falls Risk (for those patients who are an Extreme Falls Risk and as per the *Frequent Faller and Further Injury Prevention Guideline*)
- Altered conscious state (e.g. confusion, restlessness, aggressive tendencies, wandering patients, post-ictal) and acute mental health condition or on forms under the Mental Health Act 2014.
- Being transferred to units where clinical support staff are not available.
- If a patient has been administered Cytotoxic medication and is incontinent or at risk of blood spill
- Patients, who cannot speak English, are deaf or who have communication difficulties. Use accredited language or AusLan interpreters where possible
- Patients with an intellectual disability. Refer to OPH Policy *Consent to Treatment* for more information

Appendix 7: Checklist For paperwork management for interhospital transfer within SCGOPHCG (02/07/2025 – 27/08/2025)

Appendix II: Guide for paperwork management for interhospital patient transfers within SCGOPHCG- read/write				
Interhospital transfer Patient paperwork management checklist	OPH – SCGH ED – Admitted to SCGH	OPH -SCGH ED (Treat and Return)	OPH (write) – SCGH (read)	SCGH (read) - OPH (write)
Nursing Clinical Handover form	Send with patient to SCGH ED	Send with patient to SCGH ED	Send with patient	Send with patient
Complete Admission paperwork	Send OPH admission with patient when transferred to SCGH Emergency	All OPH admission paperwork is returned to OPH with patient.	Send OPH admission with patient when transferred to SCGH ward.	SCGH admission is filled in to SCGH medical record and sent with the patient to OPH
ED paperwork	N/A	SCGH ED paperwork sent back to OPH with patient. OPH Ward Clerk prepped SCGH ED paperwork and sends to OPH Medical Records for Priority Scanning	N/A	OPH admission paperwork sent with patient
Anticoagulation Medication Chart	New forms started in SCGH ED. On admission to SCGH ward: - These forms are photocopied on arrival to SCGH, by Clerk or Clinical staff. - The photocopies are stamped as “COPY” and placed with the OPH admission paperwork. - Original OPH Critical Co Use forms that continue to be used at SCGH are relabelled with SCGH admission DMR labels. - OPH admission paperwork is returned to OPH Medical Records via SCGH Medical record within 24 hours of patient arriving at SCGH. OPH admission paperwork is priority scanned at OPH	If new form started at SCGH ED- form is photocopied, the photocopy is stamped as copy and sent for priority scanning with ED paperwork. The Original form is relabelled with OPH admission label.	- These forms are photocopied on arrival to SCGH, by Clerk or Clinical staff. - The photocopies are stamped as “COPY” and placed with the OPH admission paperwork. - Original OPH Critical Co Use forms that continue to be used at SCGH are relabelled with SCGH admission DMR labels. - OPH admission paperwork is returned to OPH Medical Records via SCGH Medical record within 24 hours of patient arriving at SCGH. - OPH admission paperwork is priority scanned at OPH	- These forms are photocopied on arrival to OPH by Clerk or Clinical staff. - The photocopies are stamped as “COPY” and filed In the SCGH admission paperwork. - Original SCGH Critical Co Use forms that continue to be used at OPH are relabelled with OPH admission DMR labels. - SCGH Medical Record is return to SCGH when the patient is discharged from OPH.
WA Hospital Medication Chart – Short Stay				
WA Hospital Medication Chart – Long Stay				
Insulin Subcutaneous Order & Blood Glucose Record- Adult				
Medication History & Management Plan				
Nursing Admission Assessment				
Comprehensive Care plan				
Closed paper-based record	OPH closed medical record and SCGH open medical record are sent with patient.			

		OPH and SCGH closed medical records are sent with patient.	OPH closed medical record (if available) is sent with patient when their care is transferred. The medical records are return to OPH on discharge or when no longer required.	SCGH medical record remains open and all SCGH paperwork continues to be filed in it until SCGH go live 7am 27 th of August 2025 OPH medical record is closed from 7am on the 2 nd of July. All paper is scanned to the DMR from this date. No paperwork dated post go live is files in the OPH Hard Copy Medical Record.
OPH/SCGH Admission Paperwork	ED will file ED paperwork into SCGH medical record. SCGH medical record, OPH admission paperwork and any closed OPH medical record (if available) is sent to the receiving ward at SCGH.	ED will file ED paperwork into SCGH medical record. SCGH medical record, OPH admission paperwork and any closed OPH medical record (if available) is sent back with the patient to OPH	All admission paperwork and any closed hard copy Medical Records with the patient.	SCGH admission paperwork is filed in SCGH Medical Record. OPH admission paperwork is sent to OPH Medical Record for scanning to the DMR.
How to secure documents for transport between sites	All documentation, OPH closed medical record (if available) and SCGH medical record are transporting in OPH Transport Satchel (Image 1)	All documentation and closed medical records are transporting in OPH Transport Satchel (Image 1)	Use OPH Transport Satchel (Image 1) or SCGH Yellow internal envelope.	SCGH Medical Record is transported via a Yellow Internal Mail envelope.